

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Gardasil® 9 for vaccination against Human Papillomavirus (HPV)

Summary of NICE / NICE CKS Guidelines for HPV Vaccination

Overview

- HPV is a common sexually transmitted infection.
- Persistent infection with high-risk HPV types is linked to:
 - o **Cervical cancer** (over 99% of cases)
 - o Other anogenital cancers (e.g. vulval, vaginal, penile, anal)
 - o **Oropharyngeal cancers**
 - o **Genital warts** (caused by HPV types 6 and 11)

Vaccines Used in the UK

- **Gardasil® 9** (nonavalent) — protects against 9 HPV types: 6, 11, 16, 18, 31, 33, 45, 52, 58.
- **Administered intramuscularly**, usually in the **deltoid**.

Eligibility (UK National Immunisation Programme)

Routine Adolescent Programme

- **All children aged 12–13 years** (school year 8 in England).
- **One-dose schedule** since September 2023 for most immunocompetent adolescents under 25.

Catch-Up Programme

- Available up to age **25** for those who missed the vaccine in school (especially MSM and immunocompromised individuals).

Special Risk Groups

- **Men who have sex with men (MSM)** aged up to 45 years.
- **Trans women** (if risk comparable to MSM).
- **HIV-positive individuals** or those who are **immunosuppressed**:
 - Require **two or three doses** depending on age and immune status.

Vaccination Schedules

Group	Schedule
Healthy individuals <25 years	1 dose
Aged ≥25 years	2 doses , 6–24 months apart
Immunosuppressed	3 doses (0, 1–2, and 6 months)

Contraindications

- Severe allergic reaction to previous dose or vaccine components (e.g. yeast proteins).
- Acute severe febrile illness (defer until recovered).

Side Effects

- Mild and self-limiting:
 - Pain/redness/swelling at injection site
 - Headache, nausea, fatigue
- Rare: hypersensitivity reactions, syncope (observe post-vaccination)

Effectiveness

- Highly effective at preventing:
 - **HPV-related cancers**
 - **Genital warts**
- Early vaccination (before sexual debut) provides the best protection.

Documentation

- Record **vaccine name, batch number, dose, site, and date**.
- Report any suspected adverse effects via the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Gardasil® 9

for vaccination against

Human Papillomavirus (HPV)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Gardasil® 9 is indicated for the active immunisation of individuals from 9 years of age for the prevention of premalignant genital lesions, cancers affecting the cervix, vulva, vagina and anus, and external genital warts caused by human papillomavirus (HPV) types 6, 11, 16, 18, 31, 33, 45, 52, and 58.
Inclusion criteria	- Individuals aged 9 years and over. - Individuals eligible for vaccination as part of the national programme may be vaccinated under this PGD as long as they are aware they are entitled to receive the vaccine free from an NHS service provider. The NHS eligibility criteria for the HPV vaccine is: • all adolescents (boys and girls) in school Year 8 (usually aged 12 and 13) • GBMSM (Gay, Bisexual and other Men who have Sex with Men) up to and including 45 years of age attending specialist SHSs and/or HIV clinics regardless of risk, sexual behaviour or disease status • Girls remain eligible to receive the vaccine up to their 25th birthday, and boys in the eligible cohort (born on/after 1 September 2006) remain eligible to receive the vaccine until their 25th birthday • Males and females moving to the UK from overseas who have not been offered protection against HPV in their country of origin and who meet the eligibility criteria for HPV vaccine should be offered vaccine. This would include both females born on/after 1 September 1991 and males born on/after 1 September 2006 if they are under 25 years of age, and GBMSM attending specialist SHSs up to 45 years of age. - Informed consent obtained or parental/guardian consent as appropriate. - No contraindications to HPV vaccination.
Exclusion criteria	- Previous anaphylactic or hypersensitivity reaction to any HPV vaccine or any components of the vaccine. - Individuals who have completed a course of an HPV vaccine - Pregnancy - Acute febrile illness (vaccination should be postponed until recovered).
Cautions	- Observe for 15 minutes post-vaccination due to risk of fainting. - Ensure correct schedule and age-specific regimen is followed. - Advise patient on common side effects and when to seek medical attention. - Record batch number and expiry date as per protocol. - Vaccination should be postponed until completion of pregnancy. If high-risk sexual activity continues during pregnancy, the patient should be referred to their GP or midwife. - The immunogenicity of the vaccine could be reduced in immunosuppressed subjects. Vaccination should proceed in accordance with the national recommendations (see Green Book chapter 18a) - Advise patient that appropriate precautions against sexually

	transmitted diseases should still continue to be used.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Gardasil 9 suspension for injection
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light. Gardasil 9 should be administered as soon as possible after being removed from the refrigerator.
Route/method of administration	• Intramuscular injection, preferably in the deltoid muscle. • Gardasil 9 may appear as a clear liquid with a white precipitate prior to agitation. Shake well before use to make a suspension. After thorough agitation, it is a white, cloudy liquid. • Gardasil 9 must not be injected intravascularly, subcutaneously or intradermally. The vaccine should not be mixed in the same syringe with any other vaccines and solution. • Gardasil 9 is an inactivated vaccine and will not be affected by, nor interfere with other inactivated or live vaccines given at the same time, or at any interval from each other. When administering at the same time as other vaccines, they should be given at separate sites, preferably in different limbs. If given in the same limb, they should be given at least 2.5cm apart.
Dose and frequency	• a 1 dose schedule for the routine adolescent programme and GBMSM programme for eligible individuals less than 25 years of age • a 2 dose schedule from the age of 25 years for the GBMSM programme (0, 6 to 24 month schedule) • a 3 dose schedule for eligible individuals who are immunosuppressed and those known to be living with HIV, including those on antiretroviral therapy (0, 1, to 6 month schedule)
Quantity to be administered and/or supplied	0.5 mL per dose according to schedule.
Maximum or minimum treatment period	• When Gardasil 9 is administered according to a 2-dose (0, 6 – 12 months) schedule. The second dose should be administered between 5 and 13 months after the first dose. If the second vaccine dose is administered earlier than 5 months after the first dose, a third dose should always be administered. • When Gardasil 9 is administered according to a 3-dose (0, 2, 6 months) schedule. The second dose should be administered at least one month after the first dose, and the third dose should be administered at least 3 months after the second dose. All three doses should be given within a 1-year period. • If the course is interrupted, it should be resumed but not repeated,

	ideally allowing the appropriate interval between the remaining doses
Adverse effects	• Injection site reactions (pain, swelling, redness), headache, fever, nausea, dizziness. Rare: hypersensitivity reactions. Refer to SmPC. • As with all injectable vaccines, appropriate medical treatment and supervision should always be readily available in case of rare anaphylactic reactions following the administration of the vaccine. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline, and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. • Syncope (fainting), sometimes associated with falling, can occur following, or even before, any vaccination, especially in adolescents as a psychogenic response to the needle injection. It is important that procedures are in place to avoid injury from fainting. • This vaccine should be given with caution to individuals with thrombocytopaenia or any coagulation disorder because bleeding may occur following an intramuscular administration in these individuals.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	• To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. • Serious side effects are rare, however in the event of systemic upset, acute illness or any other clinical concern, the patient should be referred to a clinician for prompt assessment • Advise that individuals should continue to take appropriate precautions to protect themselves from sexually transmitted diseases and unwanted pregnancy • Vaccination is not a substitute for routine cervical screening. Since no vaccine is 100 % effective and Gardasil 9 will not provide protection against every HPV type, or against HPV infections present at the time of vaccination, routine cervical screening remains critically important and should follow local recommendations. • If appropriate, advise patient/carers when the next dose is due

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
16/2/26		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		16/2/26
Chris Pilkington	Pharmacist GPhC: 2046322		16/2/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	16/2/26